MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 14 OF 28

MFT 714

Trauma-Informed Care

15 teaching weeks · 1 practicals · 0 worked cases · 5 figures

SEMESTER TWO

1.71 units · Prerequisite: MFT 712

> “Trauma changes the body, the memory, the attachment system and the family.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 714 — Trauma-Informed Care

MSc Marriage and Family Therapy · Semester Two · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 712

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	**Trauma-Informed Care**	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Trauma changes the body, the memory, the attachment system and the family. This course teaches the physiology, the phased treatment models, and the organisational stance — and it teaches you to avoid the two commonest errors: doing too much too soon, and doing nothing at all.

How to use this book

This is the most personally demanding book in the programme. Announce it in advance to your peers; know your own history; use supervision; and do not read the trauma chapters late at night. The material is presented in the order treatment unfolds: safety, then processing, then integration.

1. Course description

Trauma-informed care means organising practice around the prevalence and impact of trauma; **trauma- specific** care means delivering interventions designed to treat it. You need both.

The course covers: definitions and types of trauma; the Adverse Childhood Experiences evidence; the neurobiology of threat and regulation; dissociation; attachment and developmental trauma; the phased model; specific modalities (TF-CBT, EMDR, narrative exposure, somatic approaches); family and couple work with trauma; intergenerational transmission; re-traumatisation in services; and practitioner self-protection.

2. Learning outcomes

#	Outcome	Assessed
LO1	Define trauma and distinguish types, including complex and developmental trauma	Exam
LO2	Explain the neurobiology of threat, regulation and memory	Exam
LO3	Conduct a trauma-informed assessment without re-traumatising	Observed assessment
LO4	Apply a phased treatment model and justify the sequencing	Case study
LO5	Use at least two trauma-specific interventions under supervision	Skills test
LO6	Work systemically with trauma in couples and families	Case study
LO7	Design trauma-informed organisational practices	Report

3. Assessment

Component	Weight	Due
Trauma-informed assessment observed (40 min)	25%	Week 7

Component	Weight	Due
Phased treatment plan (2,500 words)	35%	Week 12
Trauma-informed service audit report (2,000 words)	20%	Week 14
Examination	20%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	What trauma is: single, complex, developmental, collective	Typology exercise
2	The ACE evidence and its limits	ACE discussion (with care)
3	Neurobiology I: threat, arousal and the window of tolerance	Regulation skills lab
4	Neurobiology II: memory, dissociation and the body	Grounding practice
5	Attachment and developmental trauma	Attachment-informed formulation
6	Trauma-informed assessment	Structured interview
7	**Observed assessment**	Assessed
8	The phased model: safety, processing, integration	Phase planning
9	Trauma-focused CBT with children and families	TF-CBT component practice
10	EMDR, narrative exposure and somatic approaches	Comparison and critique
11	Couples and trauma: betrayal, violence, and the repair of trust	Couple session design
12	**Treatment plan due**; intergenerational transmission	Genogram of trauma
13	Re-traumatisation in services; the four R's	Service audit
14	**Service audit due**; vicarious trauma and supervision	Personal resilience plan
15	Integration: what trauma-informed care is not	Case presentations

Week 1 — What trauma is

Definition (DSM-5/ICD-11, paraphrased). Exposure to actual or threatened death, serious injury or sexual violence, whether experienced directly, witnessed, learned about in relation to a close person, or through repeated occupational exposure.

Types.

Type	Description	Example
Single-incident	One event, adult onset	Road traffic collision, armed robbery
Complex	Repeated, prolonged, interpersonal, inescapable	Domestic abuse, childhood abuse

Type	Description	Example
Developmental	Occurs during maturation; shapes personality and attachment	Chronic neglect, chaotic caregiving
Collective	Shared by a community	War, displacement, massacre, epidemic
Vicarious / secondary	Exposure through another's trauma	Therapist, first responder, family carer

What makes an event traumatic is not only the event. It is the event plus the meaning, the interpersonal context (was it caused by someone who was supposed to protect you?), the aftermath (was the person believed? supported?), and prior vulnerability.

Complex PTSD (ICD-11) comprises three additional clusters beyond classic PTSD: affect dysregulation, negative self-concept, and disturbances in relationships. This matters clinically: classic PTSD responds well to exposure-based treatment; complex PTSD usually requires a longer stabilisation phase.

> **PRACTICAL**
> Classify five disguised cases by type, and note which additional features each has.

Week 2 — The ACE evidence

The original study. Adverse Childhood Experiences (abuse, neglect, household dysfunction) were found to have a graded relationship with later health outcomes, including depression, addiction, suicide attempts, heart disease and early death.

What it established. Early adversity is common, cumulative, and biologically embedded. It shifted the field from asking "what is wrong with you?" to "what happened to you?"

What it does not establish. That ACE scores are destiny; that any individual's future is determined; or that a high score is a diagnosis. Roughly a third of people with high ACE scores have no significant impairment — protective relationships matter enormously.

The clinical use. ACE enquiry is a conversation, not a score: "Was there anything difficult in your childhood that we should know about? You do not have to give details."

> **CAUTION**
> Asking about adversity can activate distress. Ask when you have time to respond, offer a choice, and have a grounding plan ready.

Week 3 — Neurobiology I: the window of tolerance

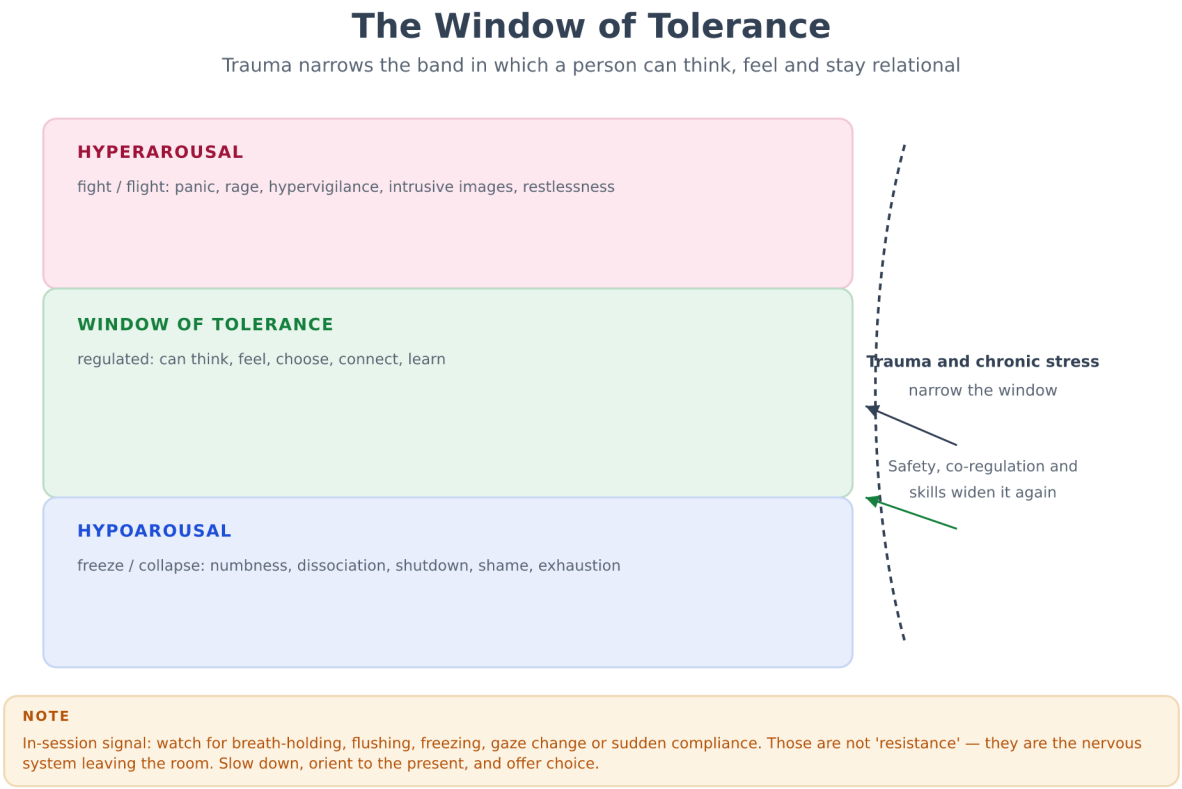


Figure 3.1 — The window of tolerance

The three states.

- **Within the window:** the person can think, feel, choose, connect and learn.
- **Hyperarousal:** fight/flight — panic, rage, hypervigilance, intrusive images, restlessness.
- **Hypoarousal:** freeze/collapse — numbness, dissociation, shutdown, shame, exhaustion.

Trauma narrows the window. Small triggers produce large reactions; the person's capacity to stay relational shrinks. This is not "over-reacting"; it is a nervous system doing what it learned.

Regulation skills to teach (and use in session).

1. **Paced breathing** — exhale longer than inhale (4 in, 6 out) for 90 seconds.
2. **Orienting** — look around the room and name five things you can see.
3. **Grounding** — feet on the floor, weight in the chair, describe the texture of an object.
4. **Movement** — stand, walk, stretch; discharge the mobilised energy.
5. **Temperature and touch** — cold water, a heavy blanket, pressure.
6. **Naming** — "I am having a memory, not the event."

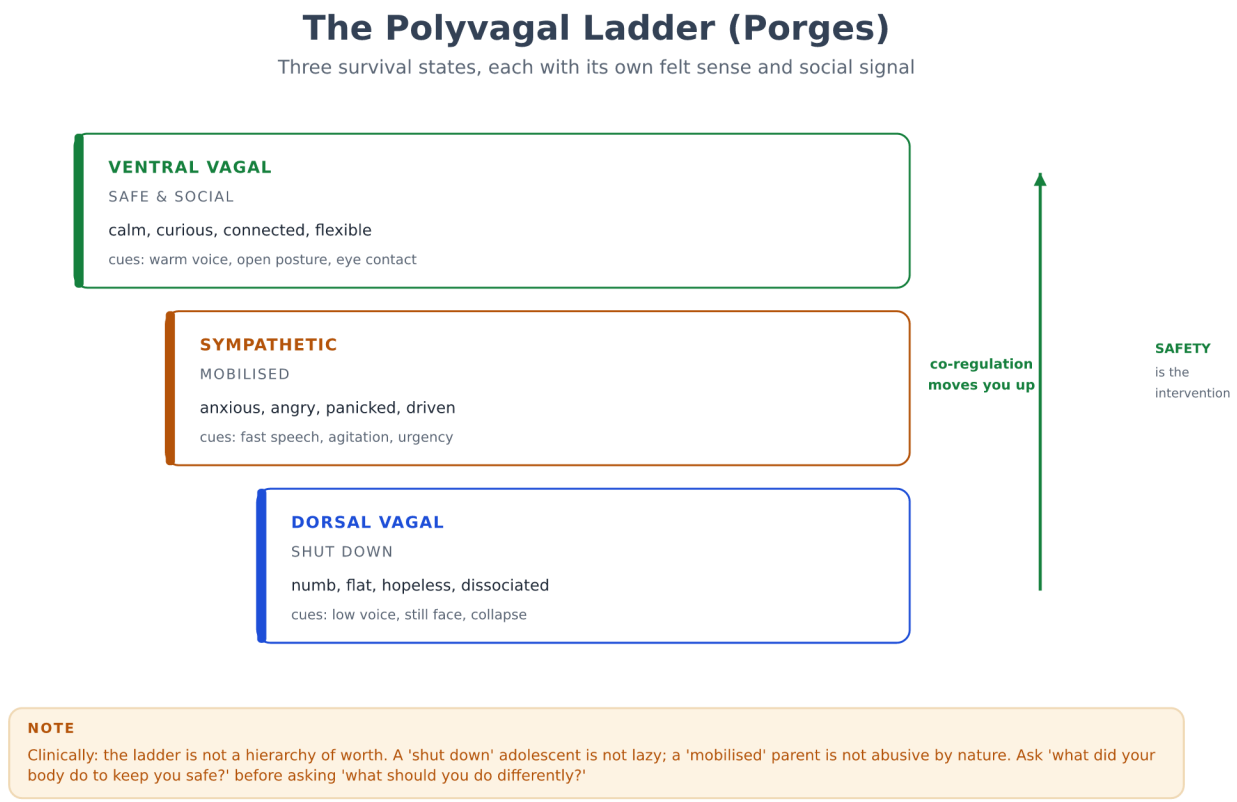


Figure 3.2 — The polyvagal ladder

In family sessions. If one member leaves the window, the session has ended for them. Pause, regulate, and return. Teaching a couple to recognise each other's early signals is itself an intervention.

Week 4 — Neurobiology II: memory, dissociation, the body

Traumatic memory is different. It is often implicit, sensory and fragmented: smells, images, body sensations, without a coherent narrative or a sense of "pastness". This is why "just talk about it" can flood a person and why containment matters.

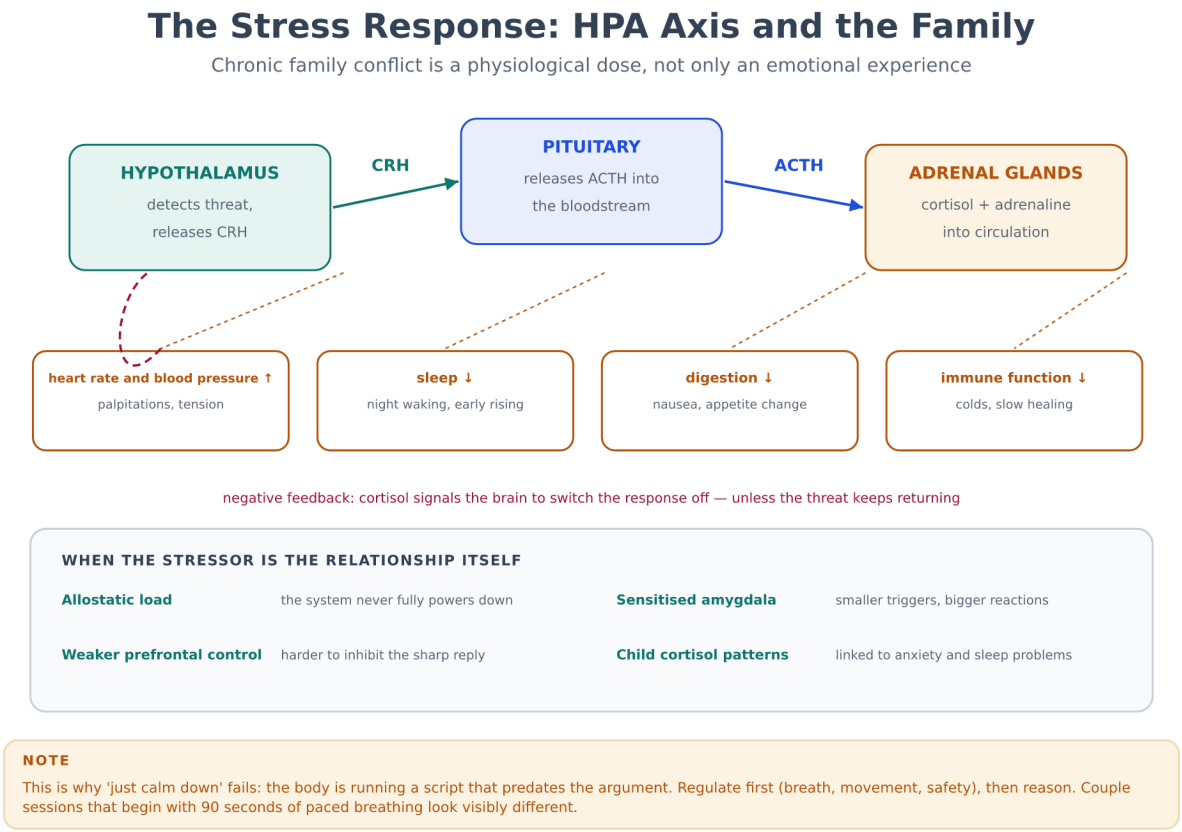


Figure 4.1 — The HPA axis and chronic stress

Dissociation — a disruption of consciousness, memory, identity or perception. Mild forms are common (going blank, feeling unreal); severe forms include depersonalisation, derealisation and dissociative amnesia. Assess gently: "Do you ever feel like you are watching yourself from outside? Do you lose time?"

The clinical rule. Do not interpret a dissociative response as resistance. If a client's eyes glaze, stop the exercise, orient them to the room, and slow down. Then shorten the next segment.

Week 5 — Attachment and developmental trauma

Developmental trauma (sometimes "complex relational trauma") occurs when the person who should provide safety is the source of danger. The child adapts with strategies that later look like personality: vigilance, people-pleasing, aggression, emotional shutdown, compulsive caregiving.

The internal working model. "I am unlovable; others are dangerous or unreliable; I must manage alone (or never let go)." These beliefs are not irrational; they were accurate in the original environment.

Clinical implications.

1. Expect the therapeutic relationship to be tested — idealisation, devaluation, testing, sudden withdrawal.
2. Predictability and reliability matter more than technique.
3. Repair after rupture is the treatment, not a distraction from it.
4. Do not interpret the transference early; be the relationship first.

Family application. Parents with their own unresolved trauma often misread their children's ordinary behaviour as threat. Psychoeducation about trauma and parenting can reduce harsh discipline dramatically.

Week 6 — Trauma-informed assessment

The four R's (SAMHSA).

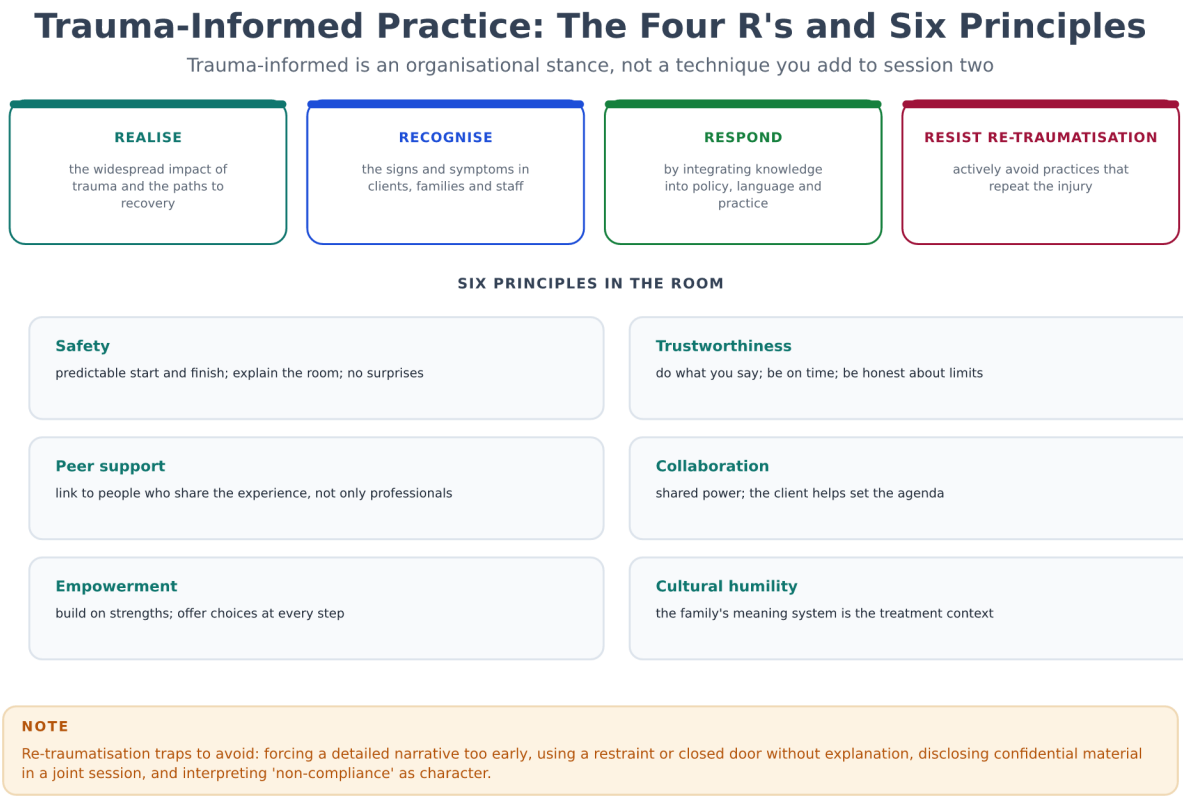


Figure 6.1 — The four R's and six principles of trauma-informed practice

1. **Realise** the widespread impact of trauma.
2. **Recognise** the signs and symptoms.
3. **Respond** by integrating knowledge into practice and policy.
4. **Resist re-traumatisation.**

Assessment principles.

- Explain why you are asking, and offer choices ("you can skip any question").

- Ask about the event only at the level of detail needed for formulation; detail belongs in processing.
- Ask about **strengths, survival strategies and meaning**, not only symptoms.
- Screen for dissociation before any trauma-focused work.
- Screen for current safety (violence, self-harm, substance use).

Instruments. PCL-5 (PTSD symptoms), IES-R, CTQ (childhood trauma; use with care), DES-II (dissociation), PHQ-9, AUDIT. Always follow up a positive screen with a conversation.

Week 7 — Observed trauma-informed assessment

Forty minutes. Assessed on: safety and choice throughout, pacing, grounding as needed, formulation without premature interpretation, and accurate risk screening.

Week 8 — The phased model

Phase 1: Safety and stabilisation. Establish safety, build the alliance, teach regulation, address practical dangers (housing, violence, substance use), and develop what Herman called "the capacity to contain".

Phase 2: Processing. Work through the traumatic material using a validated method (TF-CBT, EMDR, narrative exposure, prolonged exposure) — only when the person can tolerate it and has stabilisation skills.

Phase 3: Integration and reconnection. Rebuild identity, relationships, work, and meaning. This phase is often the longest and the most neglected.

Why sequence matters. Processing without stabilisation produces flooding, drop-out, and sometimes retraumatisation. Stabilisation without processing can produce a stable but constrained life. The judgment is clinical, not clerical: some people need two sessions of stabilisation; some need twenty.

Family application. The family's safety and stability is Phase 1 at the system level. You cannot do trauma processing in a household where the perpetrator is still present.

Week 9 — Trauma-focused CBT with children

TF-CBT (Cohen, Mannarino & Deblinger) is the best-evidenced treatment for childhood trauma. Its PRACTICE components:

Component	Content
Psychoeducation and parenting skills	Trauma responses; consistent, non-punitive discipline
Relaxation	Breathing, muscle relaxation, grounding
Affect expression and modulation	Feelings identification, coping

Component	Content
Cognitive processing	Correcting trauma-related beliefs ("it was my fault")
Trauma narrative	Gradual, structured account in the child's medium (book, drawing)
In vivo mastery	Facing avoided, safe situations
Conjoint sessions	Child shares the narrative with the caregiver
Enhancing safety	Safety skills and planning for the future

The caregiver's role is essential. The non-offending caregiver's belief, protection and support is the strongest predictor of recovery. Work with them in parallel.

Week 10 — Other trauma-specific modalities

Modality	Mechanism	Evidence	Cautions
EMDR	Bilateral stimulation during memory recall	Strong for PTSD	Requires training; not a magic technique
Narrative exposure therapy	Chronological testimony of the whole life	Strong for multiple/collective trauma	Can be destabilising without stabilisation
Prolonged exposure	Repeated imaginal and in vivo exposure	Strong	Drop-out; requires strong alliance
Somatic approaches	Body-based regulation and discharge	Emerging evidence	Beware of unfounded claims
Yoga / mindful movement	Regulation, interoception	Good adjunctive evidence	Not a substitute for trauma processing

The professional duty. Use what you are trained and supervised to use. A half-learned trauma protocol is more dangerous than a well-delivered stabilisation phase.

Week 11 — Couples and trauma

Three presentations.

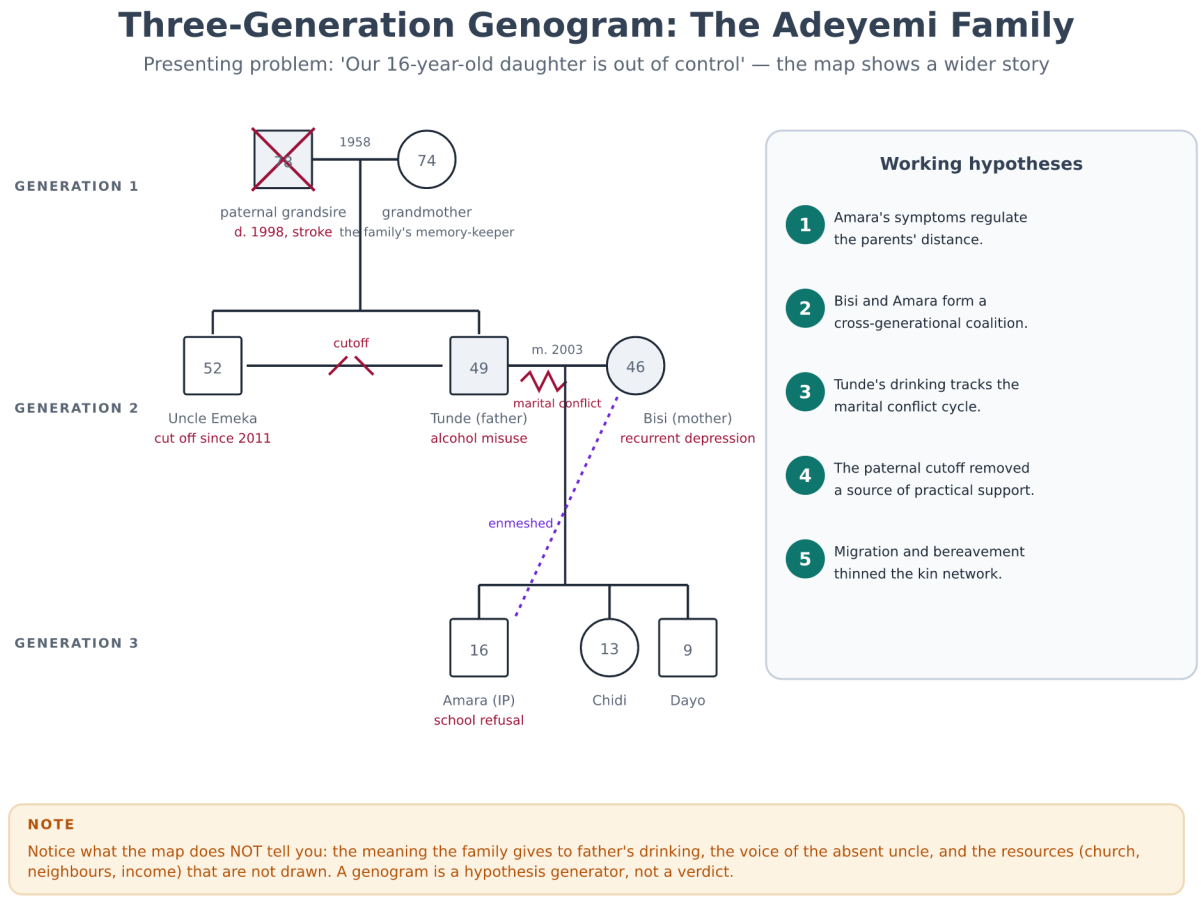
1. **One partner has a trauma history.** The other becomes a de facto carer; intimacy becomes a trigger; the couple's sexual relationship collapses. Work with both: psychoeducation for the partner, boundaries for the survivor, and a paced intimacy plan.
2. **The trauma happened inside the relationship** (betrayal, violence, coercion). This is betrayal trauma. Safety and accountability come first; joint work only where the offending partner acknowledges the harm and the victim chooses it.
3. **Both partners are trauma survivors.** Two dysregulated nervous systems in one room. The pace must be slower, the structure tighter, and the regulation explicit.

The couple-level formulation. "When she freezes, he experiences rejection and pursues; when he pursues, she freezes harder. The loop is a collision of two survival strategies, not a failure of love."

Week 12 — Intergenerational transmission

Mechanisms. Parenting behaviour (frightening, inconsistent, harsh), attachment patterns, family narrative, epigenetic and stress-physiology effects, and structural factors (poverty, displacement, community violence).

The genogram of trauma. Draw three generations and mark losses, violence, separation, migration, untimely deaths and secrets. Then ask: what was never spoken about? What did each generation have to survive?



NOTE

Notice what the map does NOT tell you: the meaning the family gives to father's drinking, the voice of the absent uncle, and the resources (church, neighbours, income) that are not drawn. A genogram is a hypothesis generator, not a verdict.

Figure 12.1 — A three-generation genogram as a map of transmission

The hopeful finding. Transmission is probabilistic, not deterministic. A consistent, protective relationship at any point breaks the chain. Your therapy may be that relationship.

Week 13 — Re-traumatisation in services

How services re-traumatise, usually without meaning to.

Practice	Why it harms	Alternative
Restraint without explanation	Replicates powerlessness	Explain, offer choices, debrief
Closed doors, no warning	Replicates entrapment	Predictability, warnings, exits
Being asked to repeat the story to every professional	Replicates helplessness	Record once, share with consent
Long waits in public areas	Replicates exposure	Private, quiet space
Disclosing in front of a partner	Replicates danger	Always screen individually
"Non-compliance" language	Replicates blame	"What got in the way?"

The organisational standard. Trauma-informed care is a policy and management issue: staff training, physical environment, restraint policy, complaint routes, and supervision. A trauma-informed clinician in a trauma-organised service will burn out.

Week 14 — Vicarious trauma

Distinguish three concepts. Burnout (exhaustion from workload), compassion fatigue (reduced empathy), and vicarious trauma (changes to the therapist's beliefs and inner world from exposure to others' trauma).

Signs. Intrusion of clients' material, nightmares, cynicism, loss of hope, changed worldview ("the world is unsafe"), avoidance of trauma cases, over-involvement.

Protection. Supervision that addresses the work's emotional impact; caseload diversity; clear boundaries; your own therapy; sleep, exercise and relationships; and acknowledging meaning — many clinicians find that sustained trauma work deepens their sense of purpose when it is properly supported.

Week 15 — What trauma-informed care is not

It is not the avoidance of difficult material. It is not an excuse for never processing trauma. It is not a synonym for "be gentle". It is a disciplined sequencing of safety, processing and integration, delivered in a relationship and an organisation that do not repeat the injury.

Glossary

Adverse childhood experiences (ACEs) — potentially traumatic events in childhood, measured cumulatively. **Betrayal trauma** — trauma caused by someone on whom the person depends. **Complex PTSD** — PTSD plus affect dysregulation, negative self-concept and relational disturbance. **Dissociation** — disruption of consciousness, memory, identity or perception. **Grounding** — techniques that return attention to the present and the body. **Re-traumatisation** — repetition of the dynamics of the original trauma in the therapy or service. **Stabilisation** — the first phase of trauma treatment, focused on safety and regulation. **Window of tolerance** — the arousal range within which a person can think and connect.

Self-assessment

Multiple choice

1. Complex PTSD is distinguished by: (a) one event (b) affect dysregulation, negative self-concept, relational disturbance (c) psychosis (d) amnesia only

2. The first phase of trauma treatment is: (a) processing (b) safety and stabilisation (c) integration (d) exposure

3. Dissociation should first be: (a) interpreted (b) assessed and accommodated (c) ignored (d) challenged

4. TF-CBT's trauma narrative is: (a) forced in session one (b) gradual and structured, in the child's medium (c) written by the therapist (d) optional and never used

5. The strongest predictor of recovery in childhood trauma is: (a) the trauma's severity (b) a believing, protective caregiver (c) IQ (d) medication

6. Vicarious trauma is: (a) the same as burnout (b) changes to the therapist's beliefs and inner world from exposure (c) a diagnosis (d) avoidable by holidays

7. Re-traumatisation in services often occurs through: (a) privacy (b) predictability (c) restraint without explanation (d) choice

8. Processing trauma without stabilisation risks: (a) faster recovery (b) flooding and drop-out (c) improved alliance (d) no effect

9. Intergenerational transmission is: (a) deterministic (b) probabilistic and can be interrupted (c) genetic only (d) irrelevant

10. A trauma-informed organisation is characterised by: (a) only individual therapy (b) policy, environment, training and supervision aligned with trauma knowledge (c) shorter sessions (d) avoiding trauma discussion

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 c · 8 b · 9 b · 10 b

Short answer

1. Distinguish complex PTSD from classic PTSD.
2. Describe three regulation skills and when you would use each.
3. Name the four R's of trauma-informed care.
4. Give the three phases of trauma treatment with the task of each.
5. List four ways services re-traumatise clients and one alternative for each.

Essay (choose one, 2,000 words)

1. "Trauma-informed care is a stance, not a technique." Discuss with reference to your own practice.
2. Design a trauma-informed family service: policies, environment, staffing and clinical pathway.
3. Analyse the challenges of trauma work with a couple where the trauma occurred inside the relationship.

Further reading

- Herman, J. L. (1992/2015). Trauma and Recovery. Basic Books.
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Next: MFT 715 — Family Systems Theory